Taking Charge of Your Health

Cancer Screening for Chinese Americans

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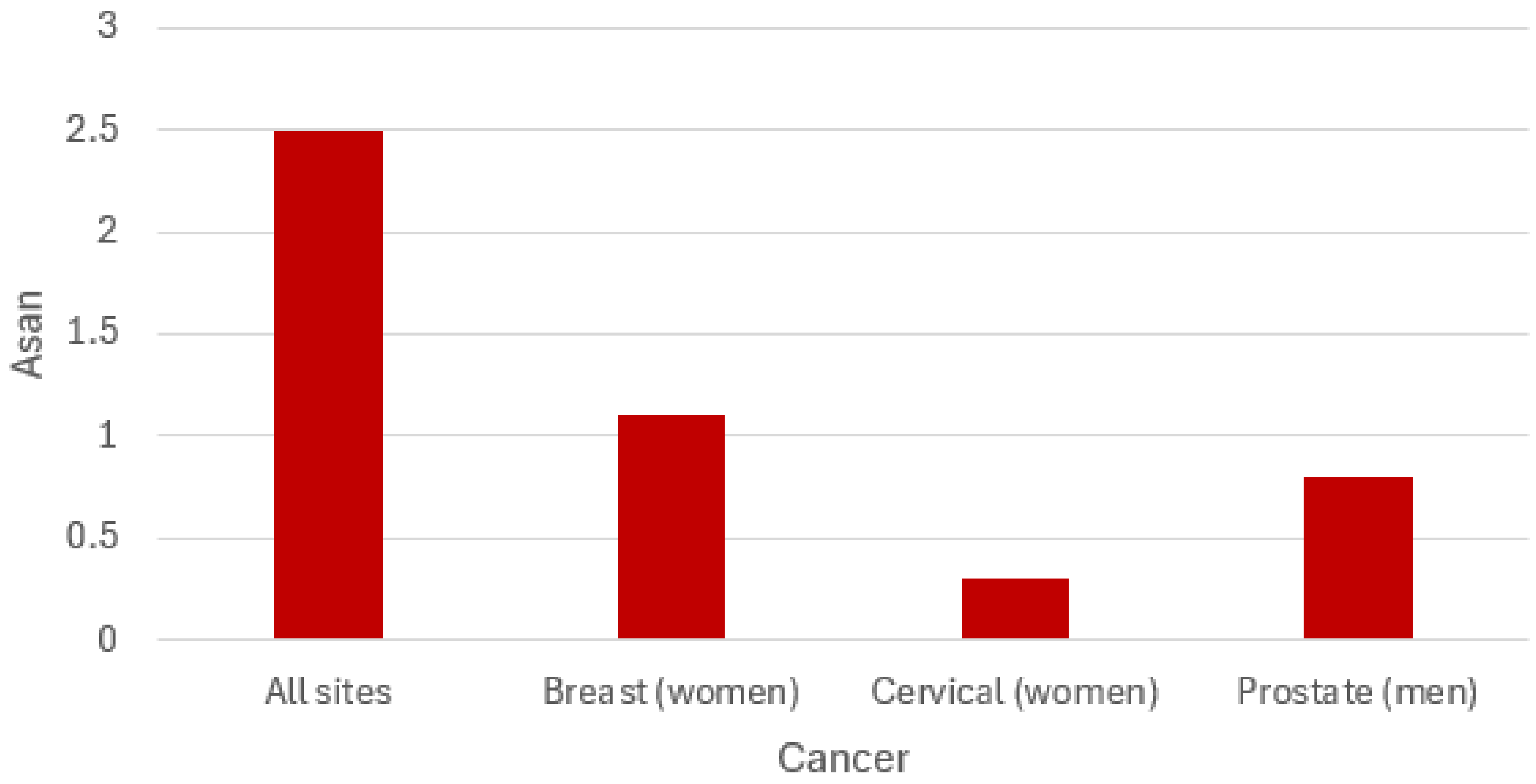
Age-adjusted percentages of cancer among adults aged 18 and over, 2019



Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

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Leading Causes of Death Among Asian Americans (2020-2021)

Leading Causes of Death Among Asian Americans (2020-2021)

		Ranking				
	All causes	**1**	**2**	**3**	**4**	**5**
Asian American						
Asian Indian	25,457	Heart diseases 5,641	Cancer 4,091	COVID-19 3,268	Cerebrovascular diseases 1,457	Accidents (unintentional injuries) 1,234
Chinese	40,489	Cancer 9,794	Heart diseases 7,791	COVID-19 4,711	Cerebrovascular diseases 2,788	Alzheimer's disease 1,509
Filipino	36,563	Cancer 7,270	Heart diseases 7,201	COVID-19 6,423	Cerebrovascular diseases 2,523	Diabetes mellitus 1,789
Japanese	18,866	Heart diseases 3,730	Cancer 3,403	Alzheimer's disease 1,706	Cerebrovascular diseases 1,506	COVID-19 1,234
Korean	16,362	Cancer 3,737	Heart diseases 2,546	COVID-19 1,991	Cerebrovascular diseases 1,083	Alzheimer's disease 762
Vietnamese	19,262	Cancer 4,366	COVID-19 2,976	Heart diseases 2,970	Cerebrovascular diseases 1,510	Accidents (unintentional injuries) 870

Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

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Most Common Cancers in Chinese-American Men

01 Lung and Bronchus Cancer (18%)

02 Prostate Cancer (16%)

03 Colon and Rectal Cancer (12%)

04 Liver Cancer (8%)

Key Takeaway: Lung and bronchus cancer is the most common among Chinese-American men.

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Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

Most Common Cancers in Chinese-American Women

01 Breast (32%)

02 Lung and Bronchus Cancer (13%)

03 Colon and Rectal Cancer (8%)

04 Thyroid Cancer (6%)

Key Takeaway: Breast cancer is the most common among Chinese-American women.

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Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

Cancer in Asian Americans - Epidemiology and Prevention

https://www.youtube.com/watch?v=j2M0tZz6LI8

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Mortality Rates of Different Cancers for Male Chinese-Americans

01 Lung and Bronchus Cancer (25%)

02 Liver and Intrahepatic Bile Cancer (11%)

03 Colon and Rectal Cancer (10%)

04 Pancreatic Cancer (8%)

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Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

Mortality Rates of Different Cancers for Female Chinese-Americans

01 Lung and Bronchus Cancer (22%)

02 Breast Cancer (12%)

03 Colon and Rectal Cancer (10%)

04 Pancreatic Cancer (9%)

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Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

Major Cancer Risk Factors

Smoking (especially among men)	Hepatitis B infection (linked to liver cancer)	Dietary habits (e.g., high salt intake)	Sedentary Lifestyle	Genetic predispositions	Alcohol consump-tion	Low screening rates and late-stage diagnosis

Key Takeaway: Healthy lifestyle choices and screenings can lower risk.

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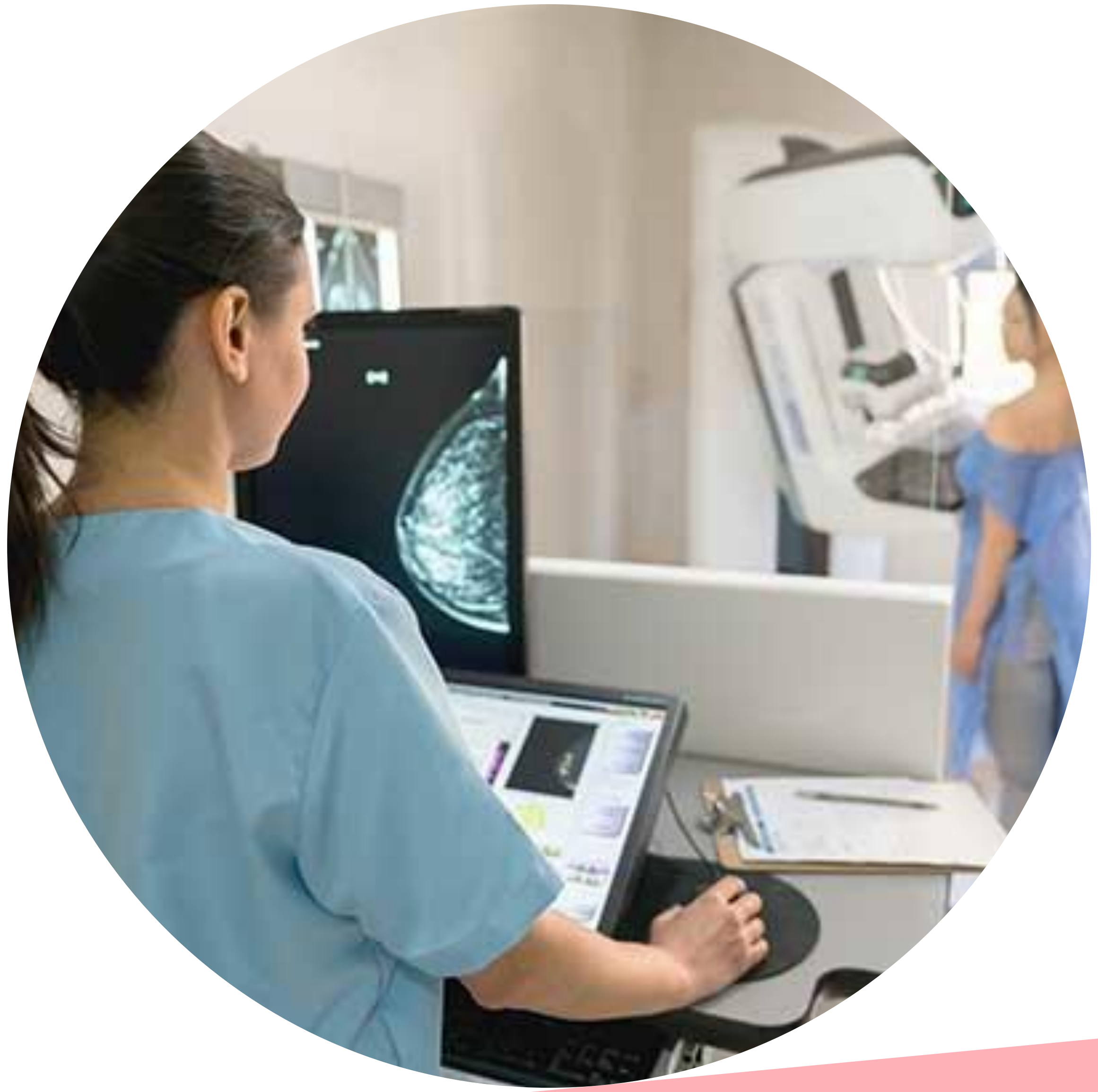

Why Cancer Screening Matters

Cancer is the #1 cause of death among Chinese Americans

Screening can detect cancer early when it is most treatable

Regular check-ups can prevent late-stage diagnosis

Early Detection Saves Lives

Key Takeaway: Regular screenings save lives.

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Why Cancer Screening Matters

A video from an accredited US hospital about the importance of early detection and screening for colorectal cancer.

https://www.youtube.com/watch?v=Ltz4Tb1I7k
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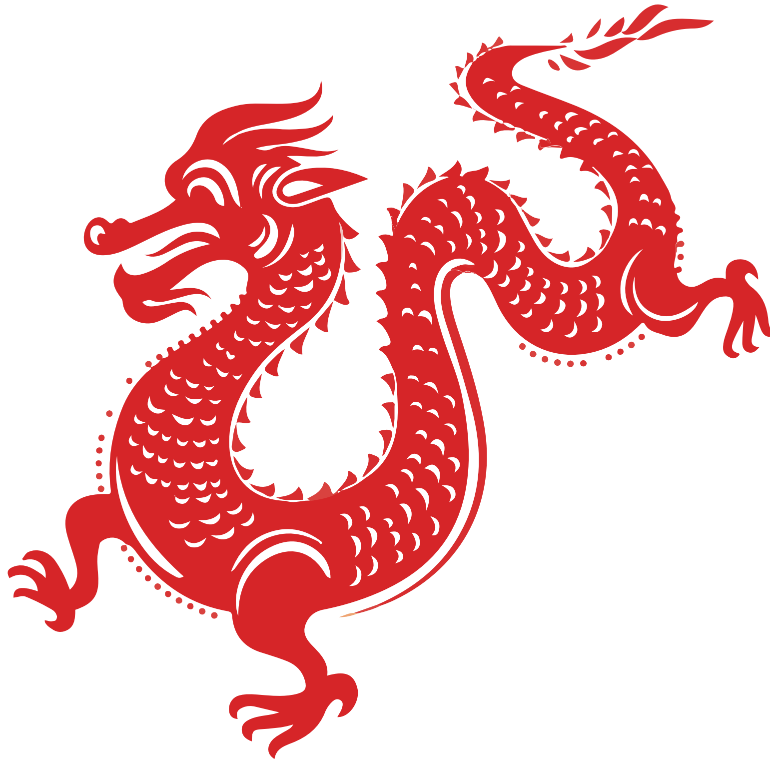
Myths vs Facts

Key Takeaway: Don’t let myths stop you from protecting your health.



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Myths vs Facts

01

Myth: "If I feel fine, I don’t need to get screened" → Fact: Many cancers have no early symptoms

02

Myth: “ I should wait until I am older to get screened.” → Fact: Everyone has different risk factors, and no two circumstances will be the same

03

Myth: "Cancer is a death sentence" → Fact: Many cancers are treatable when caught early

04

Myth: "Screening is painful or shameful" → Fact: Most tests are quick, safe, and private

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When to Get Screened

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American Cancer Society Recommendations for the Early Detection of Cancer in Average-risk Asymptomatic People*

Cancer Site	Population	Test or Procedure	Recommendation
Breast	Women, ages 40-54	Mammography	Women should have the opportunity to begin annual screening between the ages of 40 and 44. Women should undergo regular screening mammography starting at age 45. Women ages 45 to 54 should be screened annually.
	Women, ages 55+		Transition to biennial screening, or have the opportunity to continue annual screening. Continue screening as long as overall health is good and life expectancy is 10+ years.
Cervix	Women, ages 25-65	HPV DNA test, **OR** Pap test & HPV DNA test	**Preferred:** Primary HPV test alone every 5 years with an FDA-approved test for primary HPV screening. **Acceptable:** Co-testing (HPV test and Pap test) every 5 years or Pap test alone every 3 years.
	Women, ages >65		Discontinue screening if results from regular screening in the past 10 years were negative, with the most recent test within the past 5 years.
	Women who have been vaccinated against HPV		Follow age-specific screening recommendations (same as unvaccinated individuals).
	Women who have had a total hysterectomy		Individuals without a cervix and without a history of cervical cancer or a history of CIN2 or a more severe diagnosis in the past 25 years should not be screened.

Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

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Colorectal†	Men and women, ages 45+	Guaiac-based fecal occult blood test (gFOBT) with at least 50% sensitivity or fecal immunochemical test (FIT) with at least 50% sensitivity, **OR**	Annual testing of spontaneously passed stool specimens. Single stool testing during a clinician office visit is not recommended, nor are "throw in the toilet bowl" tests. In comparison with guaiac-based tests for the detection of occult blood, immunochemical tests are more patient-friendly and are likely to be equal or better in sensitivity and specificity. There is no justification for repeating FOBT in response to an initial positive finding.
		Multi-target stool DNA test, **OR**	Every 3 years
		Flexible sigmoidoscopy (FSIG), **OR**	Every 5 years alone, or consideration can be given to combining FSIG performed every 5 years with a highly sensitive gFOBT or FIT performed annually
		Colonoscopy, **OR**	Every 10 years
		CT Colonography	Every 5 years
Endometrial	Women at menopause		Women should be informed about risks and symptoms of endometrial cancer and encouraged to report unexpected bleeding to a physician.
Lung	Men and women, ages 50-80 who have a 20+ pack-year smoking history	Low-dose helical CT (LDCT)	The American Cancer Society recommends annual LDCT screening in generally healthy adults who have a 20-pack year smoking history, regardless of time since quitting if applicable.
Prostate	Men, ages 50+	Prostate-specific antigen test with or without digital rectal examination	Men who have at least a 10-year life expectancy should have an opportunity to make an informed decision with their health care provider about whether to be screened for prostate cancer, after receiving information about the potential benefits, risks, and uncertainties associated with prostate cancer screening. Prostate cancer screening should not occur without an informed decision-making process. African American men should have this conversation with their provider beginning at age 45.

Source: https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/aanhpi-cancer-facts-and-figures/aanhpi-cff.pdf

Addressing Cultural Barriers

1) Seek providers who respect and understand cultural values	2) Request educational materials in your preferred language	3) Ask for interpreters or bilingual providers if needed
4) Bring a trusted family member to appointments	5) Share your beliefs, concerns, or fears about screening	Ask questions until you understand the next steps.

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Traditional Chinese Medicine (TCM)

01 "治未病" (Zhi Wei Bing) – Treating disease before it happens

Ke SX. The principles of health, illness and treatment - The key concepts from "The Yellow Emperor's Classic of Internal Medicine". J Ayurveda Integr Med. 2023 Jan-Feb;14(1):100637. doi: 10.1016/j.jaim.2022.100637. Epub 2022 Nov 29. PMID: 36460575; PMCID: PMC10105247.

02 Balance of Yin & Yang – harmony between opposing forces

Ke SX. The principles of health, illness and treatment - The key concepts from "The Yellow Emperor's Classic of Internal Medicine". J Ayurveda Integr Med. 2023 Jan-Feb;14(1):100637. doi: 10.1016/j.jaim.2022.100637. Epub 2022 Nov 29. PMID: 36460575; PMCID: PMC10105247.

03 Healthy Lifestyle – diet, rest, exercise (tai chi, qigong)

Liang ZH, Yin DZ. Preventive treatment of traditional Chinese medicine as antistress and antiaging strategy. Rejuvenation Res. 2010 Apr-Jun;13(2-3):248-52. doi: 10.1089/rej.2009.0867. PMID: 19954331.

04 Focus on wellness rather than only treating sickness

Lyu RT, Ai YL, Yang ZQ, Wang T, Tang JY. [Research and development of new traditional Chinese medicine (TCM) for "preventive treatment of diseases" and innovation of TCM]. Zhongguo Zhong Yao Za Zhi. 2025 Jul;50(13):3589-3595. Chinese. doi: 10.19540/j.cnki.cjcmm.20250310.603. PMID: 40904140.

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Health Comes First

Take Action Today!

- **Talk to your doctor**
- **Schedule your screening**
- **Encourage family and friends**

Key Takeaway: Your health is in your hands—start today!

Testimonials:

"I avoided the doctor for years. A free screening helped find a problem early. I'm thankful!"

"I didn't think I needed screening because I eat healthy. But I still got tested – and it gave me peace of mind."

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MED 275 | Week 6: Lung Cancer Epidemiology & The Asian Health Disparity

https://www.youtube.com/watch?v=LHjnKMx3OTY

Local resources to take advantage of

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MD Anderson Cancer Center

The University of Texas

SCAN ME!

Making Cancer History®

MD Anderson provides cancer care at several convenient locations throughout the Greater Houston Area and collaborates with community hospitals and health systems nationwide through MD Anderson Cancer Network®.

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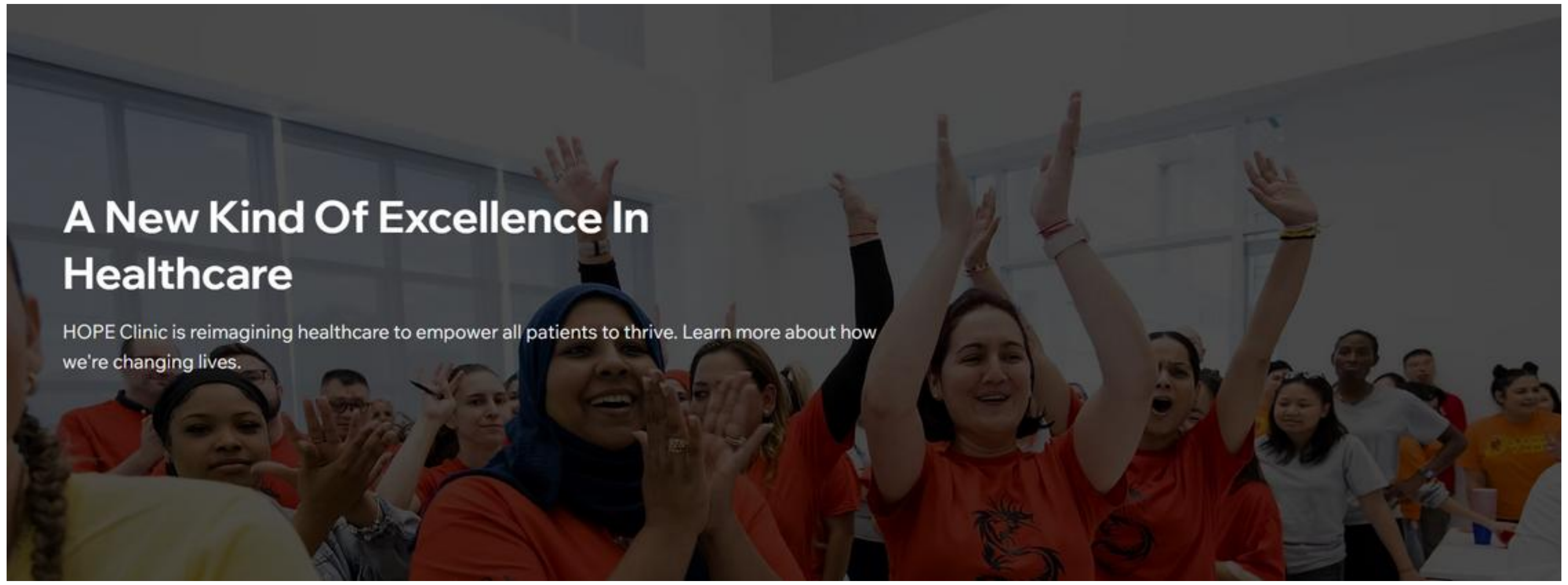
HOPE Clinic

A Community Health Center

SCAN ME!



HOPE Clinic is a full-time Federally Qualified Health Center (FQHC), serving over 20,000 unique patients with over 100,000 patient visits per year. HOPE Clinic provides health care services to all people, regardless of the patient's ability to pay. In particular, HOPE Clinic serves the uninsured, under-insured, those with limited English proficiency, and low-income patients.

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